

Wegovy

Brand-Name GLP-1 · Provider Reference

Weight Loss Program · KORB Health Group · korb-glp1-data.js v1.3 · generated 2026-08-09

Wegovy is written as a Tebra STANDARD prescription, never Tebra Compound. KORB does not manage fulfillment, does not complete prior authorizations, and does not handle coupons.

Available states

TX, CA, AL, WI

Brand-name GLP-1 through the manufacturer programs is only available in Texas, California, Alabama and Wisconsin. Source: the brand prescribing document. This is a program limit, not a shipping limit — do not offer a brand pathway to a patient outside these four states.

Rules and visit fee

Order via	Tebra Standard prescription (NOT Tebra Compound)
Billing	Patient pays the manufacturer program or the local pharmacy directly.
Prior authorizations	KORB does NOT complete insurance prior authorizations.
Coupons and savings cards	KORB does NOT handle coupons or savings cards.
Patient responsibility	KORB does not complete insurance prior authorizations and does not handle coupons or manufacturer savings cards. The patient is responsible for checking pricing, coverage and every other aspect of the medication.
Local pharmacy	A brand prescription can be sent to the patient's local pharmacy on request, under the same rules. KORB will not re-send a prescription between pharmacies to chase a lower price. The patient does that comparison before ordering.
Prescription visit fee	\$79 — charge code FITGLP1001
Fee covers	Cash-pay only. The fee covers the clinical visit, prescription issuance and program administration. Medication cost, pharmacy processing, shipping and supplies are set by the external pharmacy and paid directly by the patient.
Non-refundable	The visit fee is non-refundable once the visit has occurred or a prescription has been issued. Medication cost is not included.

Medication pricing is deliberately not listed

Manufacturer pricing changes without notice and KORB does not control it. Storing it creates a maintenance burden and risks providers quoting incorrect pricing to patients. Price and coverage are the patient's responsibility.

Wegovy — Pen

Route: subcutaneous · **Frequency:** once weekly

Order via: Tebra Standard prescription

Pharmacy: NovoCare Pharmacy, 2400 Sand Lake Road, Suite 200B, Orlando, FL 32809 · (833) 949-5527

Wegovy pens are SINGLE-DOSE, so 4 pens covers 4 weeks. This is the opposite of the Zepbound KwikPen, where ONE pen holds all four weekly doses. The Tebra entries confirm it: each Wegovy line is one strength at one volume, with no total-content figure in brackets.

Volume varies by dose. 0.25, 0.5 and 1 mg are 0.5 mL; 1.7, 2.4 and 7.2 mg are 0.75 mL. The 7.2 mg strength is listed as "Wegovy hd".

Dispensing option	Quantity	Refill	Days	When to use
4-week (28-day)	4	0	28	New prescription or any dose change. Use until the dose is stable.
8-week (56-day) — 4 pens + 1 refill	4	1	28	Clinically stable patients only.

Tebra STANDARD fields — 4-week (28-day)

Dose	Drug (dropdown)	Patient Instructions	Qty	RF	Days
0.25 mg	Wegovy 0.25 mg/0.5 ml subcutaneous pen injector	Inject 0.25 mg subcutaneously once weekly for 4 weeks.	4	0	28
0.5 mg	Wegovy 0.5 mg/0.5 ml subcutaneous pen injector	Inject 0.5 mg subcutaneously once weekly for 4 weeks.	4	0	28
1 mg	Wegovy 1 mg/0.5 ml subcutaneous pen injector	Inject 1 mg subcutaneously once weekly for 4 weeks.	4	0	28
1.7 mg	Wegovy 1.7 mg/0.75 ml subcutaneous pen injector	Inject 1.7 mg subcutaneously once weekly for 4 weeks.	4	0	28
2.4 mg	Wegovy 2.4 mg/0.75 ml subcutaneous pen injector	Inject 2.4 mg subcutaneously once weekly for 4 weeks.	4	0	28
7.2 mg [Wegovy HD]	Wegovy hd 7.2 mg/0.75 ml subcutaneous pen injector	Inject 7.2 mg subcutaneously once weekly for 4 weeks.	4	0	28

Tebra STANDARD fields — 8-week (56-day) — 4 pens + 1 refill

Dose	Drug (dropdown)	Patient Instructions	Qty	RF	Days
0.25 mg	Wegovy 0.25 mg/0.5 ml subcutaneous pen injector	Inject 0.25 mg subcutaneously once weekly for 4 weeks.	4	1	28
0.5 mg	Wegovy 0.5 mg/0.5 ml subcutaneous pen injector	Inject 0.5 mg subcutaneously once weekly for 4 weeks.	4	1	28
1 mg	Wegovy 1 mg/0.5 ml subcutaneous pen injector	Inject 1 mg subcutaneously once weekly for 4 weeks.	4	1	28
1.7 mg	Wegovy 1.7 mg/0.75 ml subcutaneous pen injector	Inject 1.7 mg subcutaneously once weekly for 4 weeks.	4	1	28
2.4 mg	Wegovy 2.4 mg/0.75 ml subcutaneous pen injector	Inject 2.4 mg subcutaneously once weekly for 4 weeks.	4	1	28
7.2 mg [Wegovy HD]	Wegovy hd 7.2 mg/0.75 ml subcutaneous pen injector	Inject 7.2 mg subcutaneously once weekly for 4 weeks.	4	1	28

- The 7.2 mg strength is marketed as Wegovy HD. Same molecule, higher strength.

- Dose escalation follows FDA-approved labeling and is individualized.

Wegovy — Oral tablet

Route: oral · **Frequency:** once daily

Order via: Tebra Standard prescription

Pharmacy: NovoCare Pharmacy, 2400 Sand Lake Road, Suite 200B, Orlando, FL 32809 · (833) 949-5527

Dispensing option	Quantity	Refill	Days	When to use
30-day supply	30	0	30	New prescription or any dose change. Use until the dose is stable.
60-day supply	60	0	60	Clinically stable patients only. Quantity 60, no refill — not 30 with a refill.

Tebra STANDARD fields — 30-day supply

Dose	Drug (dropdown)	Patient Instructions	Qty	RF	Days
1.5 mg	Wegovy 1.5 mg tablet	Take 1 tablet by mouth daily on empty stomach with up to 4 oz water. Swallow whole. No food, drink or other oral meds for 30 min.	30	0	30
4 mg	Wegovy 4 mg tablet	Take 1 tablet by mouth daily on empty stomach with up to 4 oz water. Swallow whole. No food, drink or other oral meds for 30 min.	30	0	30
9 mg	Wegovy 9 mg tablet	Take 1 tablet by mouth daily on empty stomach with up to 4 oz water. Swallow whole. No food, drink or other oral meds for 30 min.	30	0	30
25 mg	Wegovy 25 mg tablet	Take 1 tablet by mouth daily on empty stomach with up to 4 oz water. Swallow whole. No food, drink or other oral meds for 30 min.	30	0	30

Tebra STANDARD fields — 60-day supply

Dose	Drug (dropdown)	Patient Instructions	Qty	RF	Days
1.5 mg	Wegovy 1.5 mg tablet	Take 1 tablet by mouth daily on empty stomach with up to 4 oz water. Swallow whole. No food, drink or other oral meds for 30 min.	60	0	60
4 mg	Wegovy 4 mg tablet	Take 1 tablet by mouth daily on empty stomach with up to 4 oz water. Swallow whole. No food, drink or other oral meds for 30 min.	60	0	60
9 mg	Wegovy 9 mg tablet	Take 1 tablet by mouth daily on empty stomach with up to 4 oz water. Swallow whole. No food, drink or other oral meds for 30 min.	60	0	60
25 mg	Wegovy 25 mg tablet	Take 1 tablet by mouth daily on empty stomach with up to 4 oz water. Swallow whole. No food, drink or other oral meds for 30 min.	60	0	60

- Sent electronically through Tebra as a STANDARD prescription. No insurance billing and no prior authorization.

- Patient completes enrolment, payment and shipping directly with NovoCare.

- Oral semaglutide has lower and more variable bioavailability than injectable. Adherence to the empty-stomach and 30-minute rules is critical to efficacy.

- Weight loss response may be less pronounced than with injectable GLP-1 therapy.

Is this the right patient

Program: **Metabolic Health & Weight Loss Program** · Visit cadence: **Every 4 to 8 weeks**

Indications

• Weight loss
• Medically driven weight loss
• Increased energy
• Better sleep
• Improved mental health
• Improved cardiovascular health

Identifying the appropriate candidate

Body mass index	BMI greater than 25 is overweight · BMI greater than 30 is obese
FDA-approved uses	• Weight loss • Type 2 diabetes • Prevention of cardiovascular death, myocardial infarction and stroke in adults with established cardiovascular disease who are overweight or obese
Diabetic and pre-diabetic patients	KORB treats weight loss. Patients who are diabetic or pre-diabetic are acceptable. The patient must continue diabetes management with their PCP — KORB does not manage diabetes and does not order or monitor labs for it.

Agent monograph — Semaglutide — GLP-1 receptor agonist

Definition	Long-acting glucagon-like peptide-1 (GLP-1) receptor agonist. A 31-amino-acid analogue with roughly 94% homology to human GLP-1, acylated with a C18 fatty diacid that binds albumin and extends the half-life to about one week, allowing once-weekly dosing. FDA-approved as Ozempic and Rybelsus for type 2 diabetes and as Wegovy for chronic weight management.
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Mechanism

• Agonises the GLP-1 receptor.
• Glucose-dependent insulin secretion from pancreatic beta cells.
• Suppression of inappropriate glucagon release.
• Slowed gastric emptying, which prolongs satiety.
• Direct action on hypothalamic appetite centres, principally the arcuate nucleus, reducing hunger and energy intake.
• Insulin release is glucose-dependent, so hypoglycaemia risk is low as monotherapy.

Clinical evidence

• Strong randomised evidence — for the BRANDED product.
• STEP 1: mean weight loss near 15% of body weight at 68 weeks, against roughly 2.4% on placebo.
• SELECT: significant reduction in major adverse cardiovascular events in adults with overweight or obesity and established cardiovascular disease, without diabetes.
• Weight regain is common after discontinuation. Raise this at the start rather than letting it become a later surprise.

Compounded preparation

KORB dispenses a COMPOUNDED preparation, not the FDA-approved branded product. The molecule is approved; this specific formulation is not, and compounded products are not reviewed by the FDA for safety, efficacy or quality. Counsel accordingly and document that the distinction was explained.

Contraindications

Absolute — do not initiate	Cautions — evaluate before initiating
• Personal or family history of medullary thyroid carcinoma (MTC)	• History of pancreatitis — evaluate carefully before initiating
• Multiple endocrine neoplasia syndrome type 2 (MEN2)	• Gastroparesis or other significant gastrointestinal motility disorder
• Known hypersensitivity to semaglutide or any component of the formulation	• Active gallbladder disease or prior gallbladder-related surgical complications
• Pregnancy, breastfeeding, or planning pregnancy	• Severe renal impairment (eGFR below 30 mL/min/1.73 m ²)
	• Diabetic retinopathy — rapid glycaemic improvement can transiently worsen it
	• History of suicidal ideation or active depression — no causal link has been established in regulatory review, but monitor and ask

Medication interactions

• Delayed gastric emptying can alter absorption of concomitant oral medication. Use caution with narrow-therapeutic-index drugs such as levothyroxine and warfarin.
• Insulin and sulfonylureas: additive hypoglycaemia risk. Dose reduction of the concomitant agent is often required. Coordinate with the prescribing PCP.
• Alcohol may increase hypoglycaemia risk and worsen gastrointestinal side effects.
• ANAESTHESIA AND PROCEDURES: delayed gastric emptying raises aspiration risk under sedation. Anaesthesia guidance generally advises holding weekly GLP-1 therapy before an elective procedure. Tell the patient to disclose GLP-1 use to any surgeon, proceduralist or anaesthetist.
• Other GLP-1 receptor agonists: do not combine. Concomitant use is not recommended.

Side effects

Common	Less common
• Pain, redness or swelling at the injection site	• Pancreatitis
• Nausea and vomiting	• Gastroparesis
• Diarrhea	• Bowel obstruction
• Stomach pain	• Gallstone attacks
• Constipation	
• Low appetite	
• Headaches	
• Dizziness	

Monitoring

■ Weight and BMI at each visit	■ Right-upper-quadrant pain — assess for gallbladder disease
■ Ask about blood pressure — monitored by the PCP, not measured by KORB	■ Mood and mental health
■ Ask about recent HbA1c or glucose if diabetic — ordered by the PCP, not by KORB	■ Lean mass preservation — protein intake and resistance training
■ Gastrointestinal tolerance, especially through dose escalation	■ Injection site and adherence
■ Abdominal pain that is severe or radiates to the back — assess for pancreatitis	

Patient counseling

“Semaglutide works on the same pathway as a hormone your body already makes. It slows how quickly your stomach empties and reduces appetite signalling, so you feel full sooner and stay full longer. Nausea is the most common side effect and is usually worst in the first days after a dose increase. This is a compounded preparation rather than the brand-name product. Weight tends to come back if the medication stops, so we should talk about this as a long-term plan rather than a short course. Tell any surgeon or anaesthetist that you are taking this.”

- Gastrointestinal side effects are most common during dose escalation.
- Slower titration or holding at the current dose may improve tolerability.
- Instruct the patient to report persistent, severe or worsening symptoms.
- New or severe symptoms may require dose adjustment, temporary hold, or discontinuation.
- Vials are good for 28 days from first use regardless of the pharmacy BUD. Counsel the patient to discard at 28 days even if medication remains.

Chart attestation

Patient counseled on semaglutide as a GLP-1 receptor agonist for chronic weight management, including that a compounded preparation is being dispensed rather than the FDA-approved branded product. Mechanism, expected time course, gastrointestinal side effects during titration, pancreatitis and gallbladder warning signs, the need to disclose GLP-1 use before any procedure requiring sedation, likelihood of weight regain on discontinuation, and the monitoring and follow-up plan were reviewed. Patient verbalized understanding and consented to proceed.

ICD-10

DOCUMENTATION ONLY — NOT BILLING. Select the code matching the documented presentation.

Primary	Secondary
E66.9 — Obesity, unspecified	Z68.— — Body mass index (add the matching BMI code)
E66.01 — Morbid (severe) obesity due to excess calories	Z71.3 — Dietary counseling and surveillance
E66.3 — Overweight	Z72.3 — Lack of physical exercise

Escalation and discontinuation

Hard stop — discontinue immediately	Flag and reassess
• Suspected pancreatitis — severe persistent abdominal pain, often radiating to the back, with or without vomiting. Stop the medication and evaluate. • Hypersensitivity or anaphylaxis-type reaction. • Pregnancy identified or confirmed. • Medullary thyroid carcinoma or MEN2 identified at any point. • Persistent severe vomiting with dehydration or acute kidney injury.	• Gastrointestinal side effects that do not settle within two weeks of a dose increase — hold at the current dose or step back rather than pushing forward. • Right-upper-quadrant pain, or gallstone symptoms. • Rapid or excessive weight loss, or loss of lean mass. • New or worsening depression, or any mention of suicidal thoughts. • Worsening retinopathy in a diabetic patient during rapid glycaemic improvement. • Plateau with no further response at the maximum tolerated dose — reassess the plan rather than continuing indefinitely. • Patient scheduled for surgery or a procedure requiring sedation — coordinate holding.

• GLP-1 receptor agonists are not currently on the WADA prohibited list, so the athlete hard stop used in the Functional Health peptide program does not apply to this program. Confirm current WADA status before treating a tested competitive athlete.



Questions and escalation

- Pharmacy or shipping issues — Operations
- Charge codes and billing — Operations
- Clinical protocol questions — Clinical Director
- Corrections to this document — Clinical Operations

Do not improvise

- Do not substitute a pharmacy the tool marks as unavailable
- Do not alter quantity, refill or days supply
- Do not paraphrase patient instructions
- Do not quote pricing without confirming with Operations